Advanced MET exon 14 skipping lung cancer National Cancer Control Programme (NCCP) 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 · VERSION 2

Tepotinib monotherapy

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Make treatment fit into daily life

Take at the same time each day with food and swallow whole. If a dose is missed, take it when remembered on the same day unless the next dose is due within 8 hours; then skip it. Do not double a dose. Ask your pharmacist if the timing is uncertain or if you vomit; do not take a replacement without advice. If swallowing is difficult, ask about the approved water-dispersion method.

Before starting

Confirm the MET result, previous treatment, prescription, first review and contact number.

Taking your tablets

Take at the same time each day with food and swallow whole. If a dose is missed, take it when remembered on the same day unless the next dose is due within 8 hours; then skip it. Do not double a dose. Ask your pharmacist if the timing is uncertain or if you vomit; do not take a replacement without advice. If swallowing is difficult, ask about the approved water-dispersion method.

Supplies and reviews

Arrange repeat prescriptions before tablets run out. Keep blood-test and review appointments. The tablet regimen itself needs no infusion line or pump.

If treatment is held

Ask why, what must recover, when review is due and who will authorise restarting. Do not change between one and two tablets yourself.

At a change or stop

Agree scans, symptom follow-up and the medicine plan. Continue the contraception and breastfeeding precautions for at least one week after the final dose.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00823 v2 · official regimen
- EMA Tepmetko product information, updated 08/10/2026
- VISION long-term follow-up, Mazieres et al., JAMA Oncology 2023
- VISION treatment sequencing: ≥3-year follow-up, ESMO 2025 abstract 1995P
- VISION sustained-response subset, ASCO 2026 abstract 8612
- NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), oral therapy Table 5
- NCCP NCIS supportive antiemetics V8: parenteral guidance

Source reconciled; independent clinical, pharmacy and patient/PPI review pending